



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : رضا نعيم شوقي ضيف الله
المحافظة : القاهرة الحي / المدينة : المطرية
رقم البطاقة : 28205060101783 رقم التليفون : 01210244222
العنوان : المطرية
العمر (تاريخ الميلاد) : 06-05-1982 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 23-09-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: ----- التوقيع: -----
الاسم: د/ندى حسن الوظيفة: ----- التوقيع: -----
الاسم: د/نرمين رمضان الوظيفة: ----- التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: رضا نعيم شوقي ضيف الله National ID: 28205060101783	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Patient Name : الأستاذة / رندة فهد يوسف
Patient No. : 28205060101783
Path No. : SR-132/2023
Sex : Female
Age : 40 Y
Referring Doc. :

Request No. : 13002545
Request Date : 15/02/2023
Verification Date : 28/02/2023
Financial Category : برنامج دار السلام - فئة الدولة
Department : Out Patient
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.
Nature of Specimen: Slide revision.

Gross Examination:

- One paraffin block labeled by (723155).
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- Histological type: Invasive duct carcinoma with focal mucinous component.
- Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+2) Grade II
- DCIS: Focal
- Angioinvasion: Could not be detected within the limits of biopsy received
- IPT (Positive control showed appropriate reactivity).
- ER: Strong nuclear reaction in 95% of tumor cells (Allred score 8/8).
- PR: Intermediate nuclear reaction in 15% of tumor cells (Allred score 4/8).
- Her2: No membranous reaction in tumor cells (score 0).
- Ki67: 10% nuclear reaction of tumor cells.

Diagnosis:

- Right breast mass, Slide revision:
- Invasive duct carcinoma, Grade II with focal DCIS and focal mucinous component.
- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10% of tumor cells.

Verified by :
Dr. Marwa mahmoud

Printing Date : 02/03/2023 10:31:39

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Printed By : marwa mohamed yousse

Patient Name : الأستاذة / رشا نعيم شوقي ضيف
Patient No. : 28205060101783
Path No. : 3518/23
Sex : Female
Age : 41 Y
Referring Doc :

Request No. : 13004843
Request Date : 14/10/2023
Verification Date : 04/11/2023
Financial Category : فئة التكلفة : Out Patient
Department :
Registered by : Ali mostafa

Pathology Report

Clinical Data:

Right breast cancer post NACT.

Nature of the specimen:

Right mastectomy and axillary clearance specimens collected at 14/10/23.

Gross Examination:

Five specimens were received :

- Axillary clearance specimen : Fibrofatty tissue piece measured 3.5x3 cm , enclosing 3 nodules up to 0.5 cm .
- Sentinel lymph node biopsy : A fibrofatty tissue piece measured 3x3 cm entangling 3 nodes up to 1 cm .

- Nipple and areola complex specimen measured 4x2 cm , free of gross indurations.

- Retroareolar disc: Fibrofatty tissue piece measured 2.5x1.5 cm

- Right wide local excision specimen measured 15x14x2 cm with no covering skin.

- Tumor extent
Number: An ill defined firm gritty greyish white tumor bedsite.

Size: 6x6x1.4cm

Skin Involvement: Free grossly

Distance from margins: 0.7 cm from deep margin (least margin)

Frozen section:

Lymph nodes : Positive (1/3) .

Retroareolar disc showed mucinous carcinoma.

Free other margins.

Microscopic examination:

Histological type: Invasive mucinous carcinoma.

Tumor cellularity: 45%.

Verified by :

Dr. Esraa magdy

Printing Date : 18/11/2023 12:39:39

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Printing By : amrwa mohamed youssef

CAO
Room No. (4) - First Floor - Building No. 983, Nile Corniche - Micro El Gishma - Cairo, Egypt
تحت إشراف د. محمد مصطفى - معمل الباثولوجي - مبنى رقم (983) - طابق الأول - مبنى رقم (4) - شارع النيل - حي الجشمة - القاهرة، مصر

Patient Name : الأستاذة / رانيا محمد شوقي شريف
Patient No. : 28205060101783
Path No. : 3518/23
Sex : Female
Age : 41 Y
Referring Doc :

Request No : 13004843
Request Date : 14/10/2023
Verification Date : 04/11/2023
Financial Category : فئة الدولة
Department : Out Patient
Registered by : Ali mostafa

Pathology Report

- Angiolinvasion: Positive
- Ductal carcinoma in situ: Focal (10%).
- Lymph node involvement: Positive (5/6).
- Resection Margins:
- Retroareolar disc showed mucinous carcinoma.
- Free nipple and areola.
- Free resected margins.
- Histological response to therapy: Pathologic partial response.

Diagnosis:

Right breast cancer post NACT, mastectomy and axillary clearance specimens:

- Invasive mucinous carcinoma.
- Tumor cellularity: 45%

Resected margins:

Retroareolar disc showed mucinous carcinoma.

Free nipple and areola.

Free resected margins.

Lymph nodes : Positive (5/6).

Pathologic partial response.

Stage : ypT3 , ypN2.

CB: 4.091 , class: III

of. Gamal Abdelatty
Menan Hossam

Verified by :

dr. esraa magdy

ting Date : 18/11/2023 12:39:50

Page 2 of 2

Printing By : marwa mohamed youss

مختبر الأمراض النسائية - مبنى (4) - طابق (4) - رقم (987) - كورنيش النيل - مبنى الأكاديمية - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Mist El Qadima - Cairo, Egypt



Patient Name: Reda Naeem Shawky Daif Allah	Date : 16/02/2023
Age : 40 Year(s)	Patient ID: 28749
Exam : C.T. ABDOMEN & PELVIS WITH CONTRAST	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

**** A reported case of breast cancer.**

1. Chest:

- Right breast mass is seen measuring about 21x52 mm.
- Right enlarged axillary lymph nodes are noted, the largest measuring about 27x15 mm.
- Clear both lung fields
- No sizable pulmonary masses, areas of consolidation or cavitory lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the dorsal vertebrae.

2. Abdomen and pelvis:

- The liver is of average size with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- No lytic or sclerotic bone lesions.

OPINION:

- No definite pulmonary or hepatic or osseous deposits.

Dr. Alaa R. Hani, M.Sc.

Dr. Amr Farouk, M.D.

Patient Name: Reda Naeem Shawky Daif Allah	Date : 16/02/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 40 Year(s)
Medical ID: 511/2023	Pat. ID: 28749

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Right breast pathologically proven IDC G II

Technique

- Bilateral MLO and CC views
- Bilateral BD MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Right breast UIQ indistinct irregular mass, with calcifications within
- Complementary ultrasound showed right breast palpable indistinct hypoechoic infiltrative mass, occupying from 11 to 2 o'clock
- Scattered right breast small indeterminate nodules
- Bilateral scattered cysts
- no left breast solid mass lesions.
- Normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LNs.
- Pathological right axillary LNs, more than 3 in number, the largest is measured 2.5cm, with cortical thickness 0.8cm
- Free right infra and supra-clavicular regions

Opinion:

- Right breast locally advanced pathologically proven malignant process with pathological axillary LNs (BIRADS 6)
- Bilateral scattered cysts

Yours truly

Signature Dr. Engy Abdallah

16/02/2023 1:07 PM



NUCLEAR MEDICINE UNIT

وحدة الطب النووي

Patient Name: Reda Naeem Shawgy,

Date:- 18-6-2023

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP;

Report:

- No definite evidence of hot or cold spots that would account for metastatic bone lesions.
- Degenertaive changes are noted in the dorso-lumbar vertebrae.
- Traumatic rib lesion is noted at the Rt. 2nd rib anteriorly for follow up.
- Both kidneys are normally seen.

Conclusion:

No definite evidence of metastatic osseous deposits. Follow up bone scan is advised.

Thanks for reference

Nuclear Medicine Unit

Dr. Huda Fathy, MD



Name: Reda Naeim

Date: 03/04/2023

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: Right IDC I with DCIS & mucinous component.

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: moderate nodular

ACR: C (heterogeneously dense)

LEFT BREAST:

- Low energy (Non contrast) images: show no masses, asymmetry or distortion or suspicious microcalcifications.
- Recombined (Post contrast) images: show, no pathological uptake.
- Normal skin / nipple / areolar complex.

RIGHT BREAST:

- Low energy (Non contrast) images : Asymmetry between two breast size with relatively small right breast
- UIQ focal asymmetrical density with overlying faint microcalcifications .
- Retracted nipple with associated mild periareolar skin thickness
- Recombined (Post contrast) images : UIQ heterogeneous marked enhancing mass lesion seen extend from retroareolar to the posterior third of the breast measures 7.1x6x7cm , it is seen surrounded by multiple enhancing small sized foci scattered throughout the whole breast parenchyma more evident at both upper quadrants.
- The whole coverage area of the enhancing mass & foci 13x7x13cm.
- Complementary ultrasound:
 1. 12-1:00 spiculated mass(index lesion)
 2. Variable sized spiculated masses are seen scattered throughout the whole breast parenchyma.
 3. Scattered simple cysts
 4. Pathologically enlarged level I discrete axillary lymph nodes, the largest measures 27x14mm with diffuse skin thickness up to 7mm& eccentric hilum.

IMPRESSION:

LEFT BREAST: Normal (BIRADS1 = normal)

RIGHT BREAST: Multicentric malignant process with finding as described (BIRADS 6= pathologically proved malignancy)

Best regards,

Dr. Marwa Ramzy